

7. Schedule of Assessments

	Screening Phase ^a	Treatment Phase ^b					End of Treatment Phase Visit	Follow-Up Phase I ^{c, u}	Follow-Up Phase II ^{c, u}
Patient Visit #	0 (baseline)	1	2 ^v	3	4 ^v	5-12	13	Q 6 months	Q 12 months
Cycle (C) 1 cycle = 28 Days	0 (baseline)	C1		C2		C3, C6, C9, C12, C15, C18, C21, C24	After C26 or early termination		
Day (D) of Cycle	-30 to 0	D1 ^d	D14 ^v	D1	D14 ^v	D1			
Time Window for Assessments (days) - Arm A		-7	+/-2	-2 ⁿ	+/-2	- 7 ⁿ	30-42	+/- 35	+/-56
Time Window for Assessments (days) - Arm B		+2/- 7	+/-2	+/-2 ⁿ	+/-2	+/- 7 ⁿ	30-42	+/-35	+/-56
Informed Consent ^e	X								
Inclusion/Exclusion Criteria	X								
Medical History & Demographic Data ^f	X								
Concomitant Medications ^g	X	X		X		X	X	X ^g	X ^g
ECOG Performance Status	X	X		X		X	X		
Physical Exam/Vital Signs ^h	X	X		X		X	X		
IP Dispensing; IP and Non-IP Drug Compliance ⁱ		X		X		X	X ^s		
Adverse Event Reporting ^j	X	X	X ^v	X	X ^v	X	X	X ^w	X ^w
Hematology ^k	X	X	X ^v	X	X ^v	X	X		
Blood Chemistry, with Liver Function Tests ^k	X	X		X		X	X		
Serum/Urine Pregnancy Test ^l	X								
HbA _{1C} ^m	X					X ^m (C6, C12, C18)	X		
Circulating cfDNA ^o		X				X (C6)	X	X (5y)	X (7y, 10y)
Disease Monitoring ^p	X	Patients will be followed per ASCO Guidelines throughout the course of the study.							
Whole Blood (Pharmacogenomic)		X		(X)					

Participating Groups and Academic Identifiers:

AFT (AFT-05), ABCSG (ABCSG 42), BIG (BIG 14-03), PrECOG (PrE0109), NSABP (B-57-I)

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Patient Visit #	0 (baseline)	1	2 ^v	3	4 ^v	5-12	13	Q 6 months	Q 12 months
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Time Window for Assessments (days) - Arm A		-7	+/-2	-2 ⁿ	+/-2	- 7 ⁿ	30-42	+/- 35	+/-56
Time Window for Assessments (days) - Arm B		+2/- 7	+/-2	+/-2 ⁿ	+/-2	+/- 7 ⁿ	30-42	+/-35	+/-56
Sample ^q									
Serum		X							
Plasma ^o		X				X (C6)	X	X (5y)	X (7y, 10y)
Tumor Tissue ^f	X	In case of local/regional ipsilateral and/or distant disease recurrences and/or contralateral breast cancers, biopsies of these lesions are strongly encouraged.							
PRO Questionnaires ^s		X		X		X (C3, C6, C12, C18, C24)		X ^s	
Adherence Questionnaires ^t				X		X (C3, C6, C12, C18, C24)		X ^t	
COVID-19 related assessment							X ^y	X ^y	X ^y

Table 2: Schedule of Assessments

- Screening Phase: All screening evaluations must be completed and reviewed to confirm that patients meet all inclusion criteria and do not meet any of the exclusion criteria before randomization.
- Treatment Phase: Non-IP (if not already started) and IP therapy must start within 7 days after randomization. All assessments should be performed prior to dosing with IP (palbociclib) or Non-IP (endocrine therapy) on the visit day, unless otherwise indicated. The Schedule of Assessments during this planned 2 year period (including a visit at 30-42 days after completion of 2 years of therapy or after trigger date for early end of treatment phase for a final safety assessment) applies to both arms. IP treatment is to be given for 2 years, which is a total of twenty-six 28-days cycles (for patients without interruptions or delays). The goal is not to administer a specific number of cycles, but to allow for completion of a cycle if it started prior two years from randomization. Patients will not be allowed to begin a new IP cycle once 2 years after start of IP treatment have elapsed.
- Follow-Up Phases I & II: Half year visits can be performed via phone calls from the site to the patient or the provider, or as in person visits; yearly visits should be in person during Follow-Up Phase I. During Follow-Up Phase II, annual patient contacts shall be performed by in person visits, or phone contacts to the patient or the provider (except FU II Visit year 7 and 10 where blood samples should be collected). See section 6.1 to make determinations how the patient will move through the Follow-Up I and Follow-Up II phases of the study for assessments, data collection, and follow-up for the study. Appendix B further describes these phases.
- If screening assessments occur within 7 days before Cycle 1 Day 1, then they may serve as the Cycle 1 Day 1 assessments also and do not need to be repeated.
- Informed consent form may be obtained greater than 30 days before randomization; it must, however, be obtained prior to any protocol required assessment (i.e., Screening Phase) which is not performed as part of local routine care.
- Medical history includes clinically significant diseases that are currently active or that were active, including major surgeries, within the previous 5 years, any cancer history (including prior cancer therapies and procedures) and reproductive status. Demographic data includes age, sex and self-reported race/ethnicity. In addition, stratification factors will be assessed.
- Any concomitant medications and treatments will be recorded from 30 days prior to randomization and up the end of the Treatment Phase (refer to section 6.1). During the Follow-Up Phase I and II, only limited information on breast and/or non-breast anti-cancer related therapies have to be recorded. Endocrine therapy is not considered concomitant medication and has to be recorded separately with detailed data capture during the Treatment Phase.

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